Head Trauma & Fractures

Assess ABCDE & GCS

If Conscious, Oriented, GCS 15/15
Hemodynamically Stable.

- Assess the need for **pain killers**
- If suspected **Fracture** → Stabilize fracture, refer to **orthopedics emergency**
- If **Cut wound** → mild bleeding →Dressing, IVF, refer to **surgical emergency**

Refer to Emergency Room if Head trauma (Adult) and:

- GCS < 13 on initial assessment.
- GCS < 15 at 2 hours post-injury.
- Suspected open or depressed skull fracture.
- Any sign of basal skull fracture (haemotympanum, 'panda' eyes, cerebrospinal fluid leakage from the ear or nose, Battle's sign).
- Post-traumatic seizure.
- Focal neurological deficit.
- More than 1 episode of vomiting.
- Adults who have experienced some loss of consciousness or amnesia since the injury.
- Age 65 years or older any history of bleeding or clotting disorders.
- Dangerous mechanism of injury (a pedestrian or cyclist struck by a motor vehicle, an occupant ejected from a motor vehicle or a fall from a height of greater than 1 meter or 5 stairs).
- More than 30 minutes' retrograde amnesia of events immediately before the head injury.

Unconscious, GCS </= 14
Unstable (bleeding/ head injury).

- **Insert 2 large bore cannula**
- Start IVF
- If **Fracture** → stabilize and refer to **General hospital Surgical emergency**

Refer to Emergency If Head Trauma (Pediatrics) and:

- GCS </= 14/ age < 3 months
- Palpable skull fracture/ skull hematoma/ severe headache
- Signs of basilar skull fracture
- Post traumatic seizure
- Bulging fontanelle
- Concern of abuse
- History of LOC
- History of vomiting
- Fall from > 1.5 m height in > 2 years/ fall from > 0.9 m height < 2 years